# Comment Submitted by Onboard AI

**Jan 5, 2026**

**Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)**

## I. Introduction

Onboard AI appreciates the opportunity to provide input in response to the Department of Health and Human Services' Request for Information on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care.

Onboard AI works with health systems and AI developers to evaluate, govern, and monitor AI tools used in clinical settings. Our experience spans enterprise AI governance committees, clinical operations, quality and safety, legal and compliance, and information security teams. Our comments are informed by direct, operational experience with how AI tools are reviewed, approved, and monitored within healthcare organizations today.

Our central observation is that AI adoption in clinical care is constrained less by technical capability and more by uncertainty about what constitutes reasonable governance, evaluation, and oversight. Health systems are willing to adopt AI, but lack confidence that their internal processes will be viewed—by regulators, accrediting bodies, or courts—as sufficient and defensible.

HHS can materially accelerate adoption by reducing this uncertainty.

## II. Executive Summary (Summary Recommendation)

HHS can most effectively accelerate AI adoption in clinical care by clarifying a minimal, risk-proportionate governance baseline—aligned with existing healthcare quality and safety practices—that enables health systems to demonstrate reasonable evaluation, accountability, and monitoring of AI tools.

This approach would:

- Reduce uncertainty for providers and developers
- Improve defensibility of organizational decision-making
- Support consistent oversight without creating new certification regimes
- Require no new statutory authority or prescriptive regulation

The goal is not to define best-in-class practices, but to articulate the **least the industry needs** to demonstrate responsible AI use.

# III. General Comment on Regulation, Reimbursement, and Research & Development

Across regulation, reimbursement, and research, HHS's most effective near-term role is not to introduce new compliance frameworks, but to clarify expectations that already exist implicitly across healthcare quality, safety, and accreditation paradigms.

Healthcare organizations are deeply familiar with demonstrating:

- Defined scope of use
- Fit-for-purpose evaluation
- Clear accountability
- Continuous monitoring and re-review

Aligning AI expectations with these existing practices would enable faster adoption while preserving patient safety and public trust.

Importantly, this approach would also benefit HHS by reducing variability in oversight, limiting hindsight-driven enforcement, and enabling more consistent interpretation across federal and state actors, without expanding regulatory burden.

# IV. Responses to Specific Questions

## 1. What are the biggest barriers to private sector innovation and adoption of AI in clinical care?

The most significant barriers include:

- Unclear governance expectations for non-medical device AI used in clinical workflows
- Inconsistent evaluation requirements across health systems
- Lack of a shared reference point for "reasonable" oversight
- Limited post-deployment monitoring infrastructure in many provider organizations

These factors lead to prolonged review cycles, duplicative effort, and risk-averse decisions even for low-risk, high-value AI use cases.  AI developers are increasingly stagnated by 12+ month AI Committee review cycles being added onto existing, onerous AI Procurement processes.

## 2. What regulatory, payment policy, or programmatic changes should HHS prioritize?

HHS should prioritize **clarifying a minimal, risk-proportionate governance baseline** for AI used in clinical care, particularly for non-medical device AI.

This could be accomplished through guidance rather than rulemaking and would require no new statutory authority or CFR changes.

At a minimum, the baseline would clarify that responsible AI adoption includes four demonstrable elements:

1. Documented intended use and clinical scope
2. Fit-for-purpose pre-deployment evaluation
3. Defined governance and accountability assignment
4. Post-deployment monitoring and re-review triggers

This baseline represents **the least the industry needs** to reduce uncertainty—not a comprehensive framework or certification. Its purpose is to provide a common reference point for reasonableness, not to impose uniform technical standards.

Clear federal articulation of this baseline would also help align state-level oversight and reduce fragmentation across jurisdictions.

## 3. For non-medical devices, what novel legal and implementation issues exist, and what role should HHS play?

Key issues include:

- Ambiguity around accountability when AI influences—but does not automate—clinical decisions
- Inconsistent approaches to indemnification and responsibility allocation
- Unclear expectations for oversight of adaptive or evolving AI systems

HHS can play a constructive role by clarifying expectations for reasonable governance, enabling courts, regulators, and organizations to assess whether appropriate care was exercised without adjudicating technical model details.

## 4. For non-medical devices, what are the most promising evaluation methods, and should HHS support them?

Promising approaches include:

**Pre-deployment**

- Use-case-specific risk assessment
- Contextual performance and bias evaluation
- Documentation of known limitations tied to intended use

**Post-deployment**

- Monitoring for performance drift, bias signals, and safety events (and a definition of what minimum ‘monitoring’ means, e.g. quarterly testing vs. continuous, real-time monitoring)
- Defined triggers for reassessment (e.g. new version releases)
- Periodic governance review

HHS support would be most impactful if focused on shared evaluation infrastructure, implementation science, and real-world monitoring approaches, rather than prescriptive testing requirements.

## 5. How can HHS best support private-sector accreditation, certification, and testing activities?

HHS can best support private-sector efforts by:

- Encouraging alignment with existing healthcare quality and safety governance models
- Avoiding creation of a single federal AI certification
- Recognizing transparent, risk-based third-party assurance activities

This approach mirrors successful models in health IT and patient safety and preserves flexibility for innovation.

## 6. Where have AI tools met or fallen short of expectations?

AI tools have met or exceeded expectations when:

- Use cases are narrowly defined
- Human oversight is preserved
- Performance is monitored over time

They have fallen short when:

- Deployed without local validation
- Treated as static products rather than managed systems
- Introduced without clear accountability structures

High-value opportunities remain in workflow optimization, documentation support, triage assistance, and operational efficiency.

## 7. Which roles or governing bodies influence AI adoption, and what are the main hurdles?

AI adoption decisions are typically influenced by:

- Enterprise AI or clinical governance committees
- Clinical champions
- IT and security leadership
- Legal and risk management functions

Primary hurdles include manual review processes, lack of institutional memory, and absence of standardized governance.

### 8. Where would enhanced interoperability accelerate AI development?

Interoperability would be most impactful for:

- Standardized AI documentation and governance artifacts
- Evaluation and monitoring outputs
- Audit logs and review records
- Repeatable FHIR-based model testing requirements

Extending interoperability beyond data exchange to include governance and assurance artifacts would meaningfully reduce duplication and speed responsible adoption.

### 9. What challenges and concerns do patients and caregivers have?

Patients and caregivers seek:

- Improved access and quality
- Reduced clinician burden
- Safer, more reliable care

Their primary concerns include opacity, bias, and loss of human oversight. Visible, auditable governance and accountability mechanisms are essential to maintaining trust.

### 10. Are there specific areas of AI research HHS should prioritize?

HHS should prioritize:

- Research on governance, accountability, and monitoring models
- Longitudinal studies of AI performance and safety
- Economic analysis of AI-driven productivity and cost impacts (importantly, including pilot/POC success rates, reasons for failures, associated costs)

# V. Closing

AI adoption in clinical care will accelerate when health systems can demonstrate—clearly, consistently, and audibly—that their governance, evaluation, and monitoring practices meet a reasonable and recognizable bar.

Importantly, this approach aligns with how healthcare organizations already demonstrate readiness and accountability under established quality and safety oversight models, including accreditation-based frameworks that emphasize defined scope, organizational accountability, and continuous monitoring rather than one-time certification. Applying these familiar principles to AI governance reduces uncertainty without introducing AI-specific exceptionalism.

By clarifying a minimal, accreditation-aligned governance baseline—without creating new certification regimes or prescriptive technical standards—HHS can reduce uncertainty for providers and developers, improve the defensibility of organizational decision-making, and enable responsible innovation across the healthcare system while preserving public trust.

Respectfully submitted,

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& the **Onboard AI** Team